VITALITY 10

Avanafil

Small-molecule PDE5 inhibitor taken by mouth; active ingredient in Stendra (US) and Spedra (EU)



CLASS	STUDIED FOR	HUMAN EVIDENCE	US STATUS
PDE5 inhibitor	Erectile dysfunction	Large trials; FDA approved	FDA approved (Stendra, generics)

STUDIED FOR

Erections	ED with diabetes	ED after prostatectomy
HUMAN + ANIMAL	HUMAN	HUMAN

HOW IT WORKS IN THE BODY

Avanafil ▶ PDE5 blocked ▶ More cGMP ▶ Muscle relaxes

- **Blocks PDE5,** the enzyme that breaks down cGMP (label)
- **No direct relaxing effect** on penile tissue by itself
- **Needs sexual stimulation** to have an effect
- **More selective** vs PDE6 and PDE1 than sildenafil in lab tests

EVIDENCE & WHAT'S KNOWN

EVIDENCE

FDA APPROVED
LEVEL 5 OF 5

- Approved for ED in 2012 (Stendra)
- Given to 2,215 men in clinical trials
- Timing trial: attempts about 15 minutes after dose

NOT YET ESTABLISHED

- Combining with other ED drugs

QUICK PRACTICAL INFO

HALF-LIFE	ROUTES STUDIED
About 5 hours (terminal)	By mouth (tablets)
PHARMACOKINETICS	**SOLD AS**
Peak in 30 to 45 minutes (median, fasted); liver (CYP3A4)	Prescription tablets (Stendra, generics)
MONITORED IN TRIALS	**SPORT (WADA)**
Blood pressure, vision and hearing changes, erections over 4 hours	Not on the 2026 list

REGULATORY

FDA	Approved for ED (2012); generic tablets 2024
EMA	Spedra authorised 2013
WADA	Not on the 2026 list

REFERENCES

1. FDA, Stendra prescribing information (DailyMed), Jul 2024
2. Goldstein et al., J Sex Med, avanafil Phase 3 trial, Apr 2012
3. WADA, 2026 Prohibited List

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